

Adrabetadex Document A: MLR Background, Core Assumptions, and Provider Demo Model

Prepared for client delivery | March 30, 2026 | Audience: MLR, compliance, medical, and access leadership

Scope. This document fixes the launch facts that are already known, identifies the access rules that are already operative in the market, and translates those inputs into a provider-facing demo model. Internal Beren materials supply product-specific launch assumptions. CMS, Medicaid, HRSA, and payer policy sources control the reimbursement rules.

Background and fixed launch facts

Adrabetadex is positioned for infantile-onset Niemann-Pick disease type C with an expected U.S. PDUFA date of August 17, 2026. The current product materials state that the launch presentation will use a 900 mg/4.5 mL vial, intrathecal administration every 2 weeks, and titration driven by audiology findings plus post-dose fatigue or ataxia. The internal commercial model uses a \$1.1 million annual gross price, a 15% gross-to-net assumption, and a 50% commercial / 50% Medicaid payer mix.

Those inputs create an immediate provider-exposure issue. The provider brief uses a \$39,000 wholesale acquisition cost per 900 mg vial. At one vial per administration and 26 administrations per year, annual WAC exposure is \$1,014,000 before procedure cost, denial cost, or working-capital friction are added.

Core input	Value used in this memo	Basis
Expected PDUFA date	August 17, 2026	Adrabetadex launch materials
Launch vial configuration	900 mg/4.5 mL	Adrabetadex PIE deck, slide 36
Administration cadence	Every 2 weeks	Adrabetadex PIE deck, slide 36
Illustrative WAC per vial	\$39,000	Provider Net Cost Model brief
Annual one-vial exposure	\$1,014,000	26 administrations x WAC
Internal annual gross price	\$1.1 million	Beren training deck, slide 50
Internal GTN assumption	15%	Beren training deck, slides 44 and 50
Internal payer mix	50% commercial / 50% Medicaid	Beren training deck, slide 44
Comparable NPC list-price anchors	Aqneursa: \$350k-\$700k; Miplyffa + miglustat: \$590k-\$1.7M	Beren training deck, slide 44

Commercial payer reality

Commercial payers already manage analogous NPC therapies through tightly defined medical-necessity frameworks. UnitedHealthcare's 2025 commercial policy for Miplyffa requires genetically confirmed NPC1/NPC2 disease, use in combination with miglustat, specialist prescribing, and reauthorization only after documented positive clinical response. The same policy expressly bars concurrent use with Aqneursa. UnitedHealthcare's 2025 commercial policy for Aqneursa uses the same structure: genetic confirmation, specialist involvement, documented benefit for renewal, and no concurrent use with Miplyffa.

Route of administration does not remove payer control. UnitedHealthcare's medication sourcing protocol requires designated specialty-pharmacy sourcing for listed outpatient drugs, the specialty pharmacy bills the plan directly for the drug, the provider bills only administration, and UnitedHealthcare denies payment for the drug if the sourcing protocol is not followed. UnitedHealthcare's 2026 site-of-care policy separately states that alternative sites of care are well accepted and outpatient hospital administration requires medical-necessity documentation, with ongoing hospital-based approvals capped at 6 months before reassessment.

Analog product	Current payer behavior	Launch implication for Adrabetadex
Miplyffa (UHC)	Genetic confirmation, must be used with miglustat, not with Aqneursa, 12-month authorization, positive response required for renewal.	Adrabetadex will enter a market that already expects genetic proof, specialist prescribing, and structured reauthorization.
Aqneursa (UHC)	Genetic confirmation, must be used with miglustat or after failure/intolerance, not with Miplyffa, positive response required for renewal.	Payers will scrutinize overlap with miglustat, Miplyffa, and other NPC therapies rather than allowing an open-ended combination stack.
Spinraza (Aetna)	Precertification, genetic documentation, baseline assessment, positive response for continuation, imaging guidance covered for intrathecal administration.	Intrathecal rare-disease drugs are reviewed as procedure-plus-drug events with baseline and continuation documentation.
Brineura (Aetna)	Diagnosis confirmation, physician experienced in intraventricular administration, continuation tied to no loss or slowed loss of ambulation.	Rare neurologic routes of administration still receive narrow clinical management and continuation rules.

Planning conclusion. Adrabetadex will launch into a medical-benefit environment that already uses genetic confirmation, specialist prescribing, reauthorization based on documented benefit, site-of-care steering, and specialty-pharmacy sourcing when the plan elects to do so.

340B, Medicaid, and dual eligible mechanics

340B is a federal outpatient-drug purchasing program for covered entities. HRSA states that the 340B ceiling price equals AMP minus URA. For a new covered outpatient drug, HRSA's manufacturer help guidance states that the launch estimate uses WAC minus the applicable rebate percentage until AMP is available. Using the standard innovator rebate proxy of 23.1% against a \$39,000 WAC produces an estimated early 340B acquisition cost of \$29,991 per vial.

340B launch estimate	Per-vial amount	Annualized at 26 administrations
Illustrative WAC	\$39,000	\$1,014,000
Estimated 340B acquisition (WAC minus 23.1%)	\$29,991	\$779,766
Acquisition reduction versus WAC	\$9,009	\$234,234

Medicaid coverage for the initial pediatric population is structurally favorable but operationally strict. EPSDT requires states to furnish medically necessary Medicaid-coverable services needed to correct and ameliorate conditions in beneficiaries under age 21. That coverage entitlement does not remove prior authorization, plan editing, or billing rules. CMS's Physician Administered Drugs program also requires NDC and utilization capture for covered outpatient physician-administered drugs for payment and rebate purposes.

Dual eligible claims follow Medicare first when the beneficiary has Medicare entitlement for the service. Medicaid then fills cost-sharing up to the state's payment limit. CMS states that services covered by both programs are first paid by Medicare, with Medicaid filling in the difference up to the state's payment limit. CMS also states that providers may not

bill Qualified Medicare Beneficiaries for Medicare Part A or Part B cost-sharing amounts, even when the state limits secondary payment.

Coverage segment	How Adrabetadex will behave operationally
Medicaid child under 21	Covered through a medically necessary pediatric pathway, but still adjudicated through state or MCO prior authorization, HCPCS/NDC claim edits, and specialty-pharmacy or site rules when applied.
340B covered entity treating Medicaid FFS	Can access lower drug acquisition cost, but must align carve-in / carve-out status with the Medicaid Exclusion File and prevent duplicate discounts.
Full dual eligible / QMB	Medicare Part B pays first for covered clinician-administered outpatient drug claims, Medicaid is secondary, and the beneficiary cannot be balance billed for Medicare cost-sharing.
Medicaid managed care	MCO policy will control authorization and claims operations even while state rebate and NDC requirements continue to shape the back-end economics.

Medicaid rebate nuance. CMS states that an Exclusively Pediatric designation reduces the minimum Medicaid rebate percentage to 17.1% of AMP for qualifying innovator drugs. That is a manufacturer net-revenue lever. It does not remove prior authorization, NDC capture, or 340B duplicate-discount controls.

Small provider demo model

This demo model uses only sourced numeric facts plus the published Medicare initial-sales benchmark. It assumes one 900 mg vial per administration and one administration every 2 weeks. It intentionally leaves local procedure revenue, anesthesia, imaging, sedation, and denial-rework cost as site-specific add-ons because those values vary by center.

Scenario	Drug acquisition per administration	Benchmark drug reimbursement	Drug spread before procedure cost	Operational meaning
Non-340B buy-and-bill	\$39,000	\$40,170 (103% of WAC)	\$1,170	Provider carries the full inventory float. One unpaid administration exceeds the entire annual Part B benchmark spread.
340B center of excellence	\$29,991	\$40,170 benchmark	\$10,179	This is the strongest unit-economics case, but it depends on valid 340B acquisition, carve-in / carve-out discipline, and audit-ready records.
Commercial white-bagging	\$0 provider drug inventory	\$0 drug reimbursement to provider; administration only	\$0	Provider loses drug spread but also avoids inventory risk. Scheduling, chain-of-custody, and missed-dose exposure remain.
Full dual eligible / QMB	\$39,000	\$40,170 total allowed; Medicare primary pays \$32,136	Up to \$1,170; patient cost-share is protected	The beneficiary cannot be billed. Secondary collection sits with Medicaid and is limited by state payment policy.

If local non-drug delivery cost exceeds \$1,170 per administration, the non-340B physician-office benchmark turns negative before denial cost is counted. The 340B benchmark remains positive by \$10,179 per administration before local procedure cost.

Sources

- Beren Therapeutics. Internal training deck, January 2026, especially slides 44 and 50.

- Adrabetadex Pre-Approval Information Exchange deck, especially slides 36, 41, and 44.
- Provider Net Cost Model Concept Brief and Launch Access Reality Addendum for Adrabetadex.
- CMS. Part B Drug Payment Limits Overview. January 2025.
- CMS. Overview of Coding & Classification Systems. Updated March 19, 2026.
- CMS / Medicaid.gov. Early and Periodic Screening, Diagnostic, and Treatment.
- CMS / Medicaid.gov. Physician Administered Drugs (PAD).
- CMS / Medicaid.gov. Seniors & Medicare and Medicaid Enrollees.
- CMS. Dually Eligible Individuals - Categories. Revised October 3, 2024.
- HRSA. How is the 340B ceiling price calculated?
- HRSA OPAIS help. New Drug Price Estimation.
- HRSA. Duplicate Discount Prohibition.
- UnitedHealthcare. Medication sourcing protocol FAQ. June 24, 2025.
- UnitedHealthcare. Provider Administered Drugs - Site of Care - Commercial Medical Benefit Drug Policy. Effective January 1, 2026.
- UnitedHealthcare. Miplyffa prior authorization / medical necessity policy. Effective April 1, 2025.
- UnitedHealthcare. Aqneursa prior authorization / medical necessity policy. Effective June 1, 2025.
- Aetna. Nusinersen (Spinraza) Medical Clinical Policy Bulletin.
- Aetna. Lysosomal Storage Disorder Treatments Medical Clinical Policy Bulletin.